

Summary of ‘Health Insurance and the Demand for Medical Care: Evidence from a Randomized Experiment’

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Summary of “Health Insurance and the Demand for Medical Care: Evidence from a Randomized Experiment” by Manning, Newhouse, Duan, Keeler, Maroub, and Leibowitz

Broad Research Question

The authors investigate the effect of cost sharing on demand for medical care.

Specific Research Question

Using the Rand Health Insurance Experiment (HIE), the authors examine how varying levels of cost sharing impacts demand. They take advantage of the differing levels of copay, ranging between 0, 25, 50, 90 percent of costs, as well as ceilings on out of pocket spending to estimate demand for medical services at these differing levels of copays and maximum dollar expenditures (MDE), to attempt to answer the question of the efficacy of this program.

Policy Implications

The policy relevance should be clear here. Being able to test programs to insure families in the context of a natural experiment is a rare but valuable opportunity. It sheds light on future programs aiding in insurance assistance for individuals who may need it.

Economic Implications/Relevance

This model is mostly relevant in its leveraging of a natural experiment, aiding in finding the effects of insurance assistance, and what the optimal level may be to encourage use of medical services, in turn encouraging better health outcomes.

For my personal relevance, the four-equation model is interesting to see as an alternative to simple ANOVA estimates.

What We Learn?

We find that the HIE program **did** have an effect on demand elasticity of medical services. For all types of outpatient care, those with higher copays had higher demand elasticity, implying less demand for the services. Further, when looking at all care, those with copays above 25% had elasticities 5% higher than those with copays less than 25%. The authors also estimate a 37-60 billion dollar welfare loss by transitioning from a 95% copay (\$1000 MDE) system to the free system, though they admit this is probably overstated due to a willful ignoring of externalities. Overall, the results of the experiment are determined to be worth the cost, with improved health outcomes reducing hospital costs being a key reason why.

Empirics

The authors use an ANOVA model and the four equation model. I honestly need to study this more to be able to comment much on the validity. However, they admit to ignoring a lot of external factors, thus allowing for unneeded variation.